

HOUSE No. 4767

The Commonwealth of Massachusetts

HOUSE OF REPRESENTATIVES, November 18, 2025.

The committee on House Ways and Means, to whom was referred the Bill requiring health care employers to develop and implement programs to prevent workplace violence (House, No. 2655), reports recommending that the same ought to pass with an amendment substituting therefor the accompanying bill (House, No. 4767).

For the committee,

AARON MICHLEWITZ.

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In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act requiring health care employers to develop and implement programs to prevent workplace violence.

Whereas, The deferred operation of this act would tend to defeat its purpose, which is to require health care employers to develop and implement programs to prevent workplace violence, therefore it is hereby declared to be an emergency law, necessary for the immediate preservation of the public safety.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1. Chapter 111 of the General Laws is hereby amended by adding the
2 following section:-

3 Section 250. (a) As used in this section, the following words shall, unless the context
4 clearly requires otherwise, have the following meanings:

5 “Bodily injury”, substantial impairment of the physical condition, including, but not
6 limited to, any burn, fracture of any bone, subdural hematoma, injury to any internal organ or
7 any injury which occurs as the result of repeated harm to any bodily function or organ, including
8 human skin.

“Employee”, an individual employed by or under contract for employment with an independent entity or a third-party vendor at a health care facility, by a health care employer, who is providing health care services at, volunteering at or participating in an educational course of instruction at a health care facility.

“Health care employer”, any individual, partnership, association, corporation, trust or any person or group of persons operating a health care facility.

“Health care facility”, a hospital licensed under section 51, the teaching hospital of the University of Massachusetts medical school established under section 1 of chapter 75, Bridgewater State Hospital established under chapter 123 that is operated by the department of correction and under contract with a private company for health services, or any state acute care facility, non-acute care facility, continuing care facility or group home operated, funded or subject to oversight by the department of public health, the department of mental health or the department of developmental services; provided, that a facility with more than 1 license or that is licensed to provide multiple services shall be considered a health care facility if the facility is licensed in at least 1 of the included categories; and provided further, that health care facilities shall not include: (i) a nursing home or rest home licensed pursuant to section 71; (ii) a clinic, mobile or portable clinic or clinic satellite licensed pursuant to section 51; (iii) a home health agency certified pursuant to section 51K; (iv) an adult day health program licensed pursuant to 105 CMR 158.00; (v) a hospice program or hospice inpatient satellite licensed pursuant to section 57D; (vi) an ambulatory surgical center licensed pursuant to section 51; (vii) a renal dialysis facility licensed pursuant to section 51; (viii) an outpatient physical therapy or speech pathology facility licensed pursuant to section 23M1/2 of chapter 112; or (ix) a nursing pool agency licensed pursuant to section 72Y.

“Serious bodily injury”, bodily injury which results in a permanent disfigurement, protracted loss or impairment of a bodily function, limb or organ or substantial risk of death.

“Workplace violence”, conduct at the work site that: (i) is an unpermitted or harmful touching of another person; (ii) is an attempt or act to use some degree of physical force on another person; or (iii) could be reasonably perceived as an intent to touch without permission, use immediate physical force or injure a particular person now or in the future, that if carried out would constitute a crime, and that causes another person to reasonably believe that the person has the intent and ability to carry out such conduct.

(b) Annually, each health care employer shall perform a facility-specific risk assessment that shall include, but shall not be limited to, the standards determined by the department, in consultation with the office of health equity. The facility-specific risk assessment shall be performed in cooperation with the employees of the health care employer and any labor organization representing the employees, examining all factors that may put any of the employees at risk of workplace violence. The factors shall include, but shall not be limited to: (i) working in public settings; (ii) guarding or maintaining property or possessions; (iii) working in high-crime areas; (iv) working late night or early morning hours; (v) working alone or in small numbers; (vi) uncontrolled public access to the workplace; (vii) working in public areas where people are in crisis; (viii) working in areas where a patient or resident may exhibit violent behavior; (ix) working in areas with known security problems; and (x) working with insufficient qualified staff in 1 or more position titles to address foreseeable risk factors.

(c) Based on the findings of the risk assessment in subsection (b), the health care employer shall develop and implement a program to minimize the danger of workplace violence

to employees, which shall include, but shall not be limited to, appropriate employee training and a system for the ongoing reporting and monitoring of incidents and situations involving violence or the risk of violence. Employee training shall include, but shall not be limited to, in addition to all employer training program policies, methods of reporting to appropriate public safety officials, bodies or agencies and processes necessary for the filing of criminal charges.

(d) Each health care employer shall develop a written plan setting forth the employer's workplace violence prevention plan. The health care employer shall make the plan available to each employee and provide the plan to any of its employees upon written request. The health care employer shall provide, upon written request, the plan to any labor organization or organizations representing any of its employees. The plan shall include, but shall not be limited to: (i) a list of those factors and circumstances that may pose a danger to employees; (ii) a description of the methods that the health care employer will use to alleviate hazards associated with each factor, including, but not limited to, employee training and any appropriate changes in job design, staffing, security, equipment or facilities; (iii) a post-incident debriefing process with affected staff; and (iv) a description of the reporting and monitoring system.

(e) Each health care employer shall designate a senior manager responsible for the development and support of an in-house crisis response team for employee victims of workplace violence. The in-house crisis response team shall implement an assaulted staff action program that shall include, but shall not be limited to, group crisis interventions, individual crisis counseling, staff victims' support groups, employee victims' family crisis intervention, peer-help and professional referrals.

(f) Any health care employer who violates this section, or any regulation promulgated by the department under authority of this section may, based on the reason for the violation and the discretion of the department, be punished by a fine of not more than \$2,000 for each offense. The department or its representative, any aggrieved employee, any interested party or any officer of any labor union or association, whether incorporated or otherwise, may file a written complaint with the district court in the jurisdiction of which the violation occurs and shall promptly notify the attorney general in writing of such complaint. The attorney general, upon determination that there is a violation of any workplace standard relative to the protection of the occupational health and safety of employees or of any standard of requirement of licensure, may order any health care facility to be closed by way of the issuance of a cease and desist order enforceable in the appropriate courts of the commonwealth.

(g) No employee shall be penalized by a health care employer in any way as a result of such employee's filing of a complaint or otherwise providing notice to the department in regard to the occupational health and safety of such employee or their fellow employees exposed to workplace violence risk factors.

(h) Each health care employer shall submit a report annually, on a form prescribed by the commissioner, of all incidents of workplace violence reported to the health care employer that occurred at the health care facility on an employee, an emergency medical technician, an ambulance operator or an ambulance attendant. The report shall be submitted to the department and the office of the district attorney for the county where the health care facility is located. Not more than 90 days after receiving the reports, the department shall make the aggregate data publicly available by county and statewide; provided, that the department shall categorize the

aggregate data by occupation and incident type. The department shall create a form that meets the requirements of this section and shall require its use by each health care employer.

SECTION 2. Chapter 149 of the General Laws is hereby amended by inserting after section 52E the following section:-

Section 52F. (a) For the purposes of this section, the following words shall, unless the context clearly indicates otherwise, have the following meanings:

“Employee”, an individual employed by a health care employer.

“Health care employer”, any individual, partnership, association, corporation, trust or any person or group of persons operating a health care facility.

“Health care facility”, a hospital licensed under section 51 of chapter 111, the teaching hospital of the University of Massachusetts medical school established under section 1 of chapter 75, Bridgewater State Hospital established under chapter 123 that is operated by the department of correction and under contract with a private company for health services, or any state acute care facility, non-acute care facility, continuing care facility or group home operated, funded or subject to oversight by the department of public health, the department of mental health or the department of developmental services; provided, that a facility with more than 1 license or that is licensed to provide multiple services, shall be considered a health care facility if the facility is licensed in at least 1 of the included categories; and provided further, that health care facilities shall not include: (i) a nursing home or rest home licensed pursuant to section 71 of chapter 111; (ii) a clinic, mobile or portable clinic, or clinic satellite licensed pursuant to section 51 of chapter 111; (iii) a home health agency certified pursuant to section 51K of chapter 111; (iv) an adult day health program licensed pursuant to 105 CMR 158.00; (v) a hospice program or hospice

inpatient satellite licensed pursuant to section 57D of chapter 111; (vi) an ambulatory surgical center licensed pursuant to section 51 of chapter 111; (vii) a renal dialysis facility licensed pursuant to section 51 of chapter 111; (viii) an outpatient physical therapy or speech pathology facility licensed pursuant to section 23M1/2 of chapter 112; and (ix) a nursing pool agency licensed pursuant to section 72Y of chapter 111.

(b) A health care employer shall permit an employee to take paid leave from work if the employee: (i) is a victim of assault or assault and battery and such assault or assault and battery causes bodily injury or serious bodily injury, as those terms are defined in section 250 of chapter 111, which occurred in the line of duty; and (ii) uses the leave to seek or obtain medical treatment, victim services or legal assistance, obtain a protective order from a court, appear in court or before a grand jury or meet with a district attorney or other law enforcement official.

(c)(1) An employee seeking leave under this section shall not be required to use annual leave, vacation leave, personal leave or sick leave available to the employee, prior to requesting or taking leave under this section. Nothing within this section shall interfere with any employee's entitlement to eligible paid family and medical leave under chapter 175M.

(2) No health care employer shall require an employee to exhaust all annual leave, vacation leave, personal leave or sick leave available to the employee prior to requesting or taking leave under this section; provided, that said paid leave shall be concurrent with paid family and medical leave under chapter 175M.

(d) A health care employer may require an employee to provide documentation evidencing that the employee is a victim of assault or assault and battery sustained in the line of duty and that the leave taken is consistent with the conditions of subsections (b) and (c). An

141 employee shall provide such documentation to the health care employer within 5 business days
142 after the health care employer requests documentation relative to the employee's absence.

143 (e) An employee seeking leave from work pursuant to subsection (b) shall provide
144 advance notice of the leave to the health care employer in accordance with the health care
145 employer's leave policy; provided, however, that if an employee is absent on an unauthorized
146 basis, the health care employer shall not take any negative action against the employee if the
147 employee, within 30 days from the unauthorized absence or within 30 days from the last
148 unauthorized absence in the instance of consecutive days of unauthorized absences, provides
149 documentation that the unauthorized absence meets the criteria of subsection (b).

150 (f) All information related to the employee's leave taken pursuant to this section shall be
151 kept confidential by the health care employer and shall not be disclosed, except to the extent that
152 disclosure is: (i) requested or consented to, in writing, by the employee; (ii) ordered to be
153 released by a court of competent jurisdiction; (iii) required by federal or state law; (iv) required
154 in the course of an investigation authorized by law enforcement, including, but not limited to, an
155 investigation by the attorney general; or (v) necessary to protect the safety of the employee or
156 others employed at the workplace.

157 (g) No health care employer shall coerce, interfere with, restrain or deny the exercise of,
158 or any attempt to exercise, any rights provided by this section or to make leave requested or
159 taken pursuant to this section contingent upon whether or not the victim maintains contact with
160 the alleged abuser.

161 (h) No health care employer shall discharge or in any other manner discriminate against
162 an employee for exercising the employee's rights under this section. An employee who takes

163 leave under this section shall not lose any employment benefit accrued prior to the date on which
164 the leave taken under this section commenced as a result of taking said leave. Upon the
165 employee's return from said leave, the employee shall be entitled to restoration to the
166 employee's original job or to an equivalent position.

167 (i) Each health care employer shall post in a conspicuous place within the health care
168 facility a notice prepared by the department indicating the rights and responsibilities provided by
169 this section. The notice shall be issued in English, Spanish, Chinese, Haitian Creole, Italian,
170 Portuguese, Vietnamese, Laotian, Khmer, Russian and any other language that is the primary
171 language of at least 10,000 or one half of 1 per cent of all residents of the commonwealth. The
172 required workplace notice shall be in English and each language other than English which is the
173 primary language of 5 or more employees or self-employed individuals of that workplace, if such
174 notice is available from the department. Each health care employer shall notify each employee
175 not later than 30 days after the beginning date of the employee's employment of the rights and
176 responsibilities provided by this section, including those related to notification requirements and
177 confidentiality.

178 (j) This section shall not be construed to exempt a health care employer from complying
179 with chapter 258B, section 14B of chapter 268 or any other general or special law or to limit the
180 rights of any employee under chapter 258B, section 14B of chapter 268 or any other general or
181 special law.

182 (k) The division of labor standards, in consultation with the attorney general, may
183 promulgate regulations to carry out this section.

SECTION 3. Section 13I of chapter 265 of the General Laws, as appearing in the 2024 Official Edition, is hereby amended by striking out, in line 5, the words “treating or transporting a person”.

SECTION 4. Said section 13I of said chapter 265, as so appearing, is hereby further amended by adding the following paragraph:-

Any emergency medical technician, ambulance operator or ambulance attendant who is the victim of assault or assault and battery while in the line of duty and who is an employee of a health care facility shall be given the option of providing the address of the health care facility at which they are employed. Any emergency medical technician, ambulance operator or ambulance attendant who is the victim of assault or assault and battery while in the line of duty and is a member in good standing of a labor organization shall be given the option of providing the address of the labor organization in which they are a member in good standing. In instances where the address of the health care facility or labor organization in which the employee is a member in good standing is used, the health care facility or labor organization shall ensure that the individual receives any documents pertaining to the assault or assault and battery within 24 hours of receipt by the health care facility or labor organization. The health care facility or labor organization shall demonstrate that it has provided any and all documentation by obtaining a signature from the individual acknowledging receipt. For purposes of this paragraph, the term “employee” and the term “health care facility” shall be as defined in section 250 of chapter 111.

SECTION 5. Said chapter 265 is hereby further amended by inserting after section 13I the following section:-

Section 13I ½. Whoever knowingly and intentionally commits an assault or an assault and battery on an employee, as defined in section 250 of chapter 111, while in the line of duty, and by such assault and battery causes bodily injury, as defined in said section 250 of said chapter 111, shall be punished by imprisonment in state prison for not more than 5 years or imprisonment in a jail or house of correction for not more than 2 and one-half years or by a fine of not less than \$500 nor more than \$5,000, or any combination of said fines and imprisonment.

Whoever knowingly and intentionally commits an assault or an assault and battery on an employee, as defined in said section 250 of said chapter 111, while in the line of duty, and by such assault and battery causes serious bodily injury, as defined in said section 250 of said chapter 111, shall be punished by imprisonment in state prison for not more than 10 years or imprisonment in a jail or house of correction for not more than 2 and one-half years or by a fine of not less than \$500 nor more than \$5,000, or any combination of said fines and imprisonment.

Any employee, as defined in section 52F of chapter 149, who is the victim of assault or assault and battery at a health care facility, as defined in section 250 of chapter 111, while in the line of duty, shall be given the option of providing the address of the health care facility where the assault or assault and battery occurred or providing the address of the labor organization in which they are a member in good standing. In instances where the address of the health care facility or labor organization in which the employee is a member in good standing is used, the health care facility or labor organization shall ensure that the individual receives any documents pertaining to the assault or assault and battery within 24 hours of receipt by the health care facility or labor organization. The health care facility or labor organization shall demonstrate that it has provided any and all documentation by obtaining a signature from the individual acknowledging receipt.

SECTION 6. The commissioner of public health, in consultation with the commissioner of correction, the commissioner of developmental services and the commissioner of mental health, shall promulgate regulations within 180 days of the effective date of this act necessary to implement and enforce the purposes of section 250 of chapter 111 of the General Laws.

SECTION 7. The division of labor standards, in consultation with the attorney general, shall promulgate regulations within 180 days of the effective date of this act necessary to implement and enforce the purposes of section 52F of chapter 149 of the General Laws.

SECTION 8. (a) Notwithstanding any general or special law or rule or regulation to the contrary, not later than 1 year of the effective date of this act, the executive office of health and human services, in coordination with the executive office of public safety and security, shall submit a report to the joint committee on health care financing, the president of the Massachusetts Health and Hospital Association, Inc., the president of the Massachusetts Nurses Association and the president of 1199 SEIU with recommendations to improve interagency data sharing, communication and collaboration between health care facilities, as defined in section 250 of chapter 111 of the General Laws, and public safety and law enforcement entities to address alternative appropriate placement for criminal justice involved patients with a mental health or behavioral health diagnosis.

(b) The report shall include, but shall not be limited to, recommendations that address the following:

(i) allowing health care facilities to access reports on individuals maintained by agencies within each department of said executive offices through a secure electronic medical record, health information exchange, or other similar software or information system connected to health

care facilities, for the purposes of improving ease of access and utilization of such data for treatment and diagnosis, and supporting integration of such data within a patient's electronic health record for purposes of treatment or diagnosis;

(ii) expansion of safe and appropriate state-operated alternative placement options for patients presenting in health care facilities in acute mental health or behavioral health crisis and for whom all reasonable clinical interventions have been unsuccessful, and other alternatives, such as transfer to a more secure hospital, are unavailable;

(iii) identifying and establishing new pathways to enter patients into the department of mental health continuing care system or similar treatment that do not require an arrest; and

(iv) any further considerations necessary to fulfill the obligations of the report.

(c) Any recommendations for interagency data sharing under this section shall address the following:

(i) limiting access to identifiable information to the minimum necessary data elements required to accomplish a specific and defined public safety or workplace violence prevention purpose;

(ii) ensuring that, whenever practicable, data exchanged between agencies is de-identified or aggregated, and that identifiable information shall be shared only when essential for treatment, diagnosis or immediate safety purposes;

(iii) prohibiting the use of identifiable information obtained pursuant to this section to initiate or support criminal, civil, regulatory, licensing or administrative actions against an individual or entity, except as expressly authorized by this act;

271 (iv) requiring appropriate safeguards, access controls, logging and auditing of all data
272 access and use;

273 (v) providing that any misuse or unauthorized access to such information shall be subject
274 to penalties, which may include enforcement under chapter 93A; and

275 (vi) preserving all rights and obligations under federal law, including but not limited to
276 the Health Insurance Portability and Accountability Act of 1996, 42 CFR Part 2 and any other
277 applicable federal confidentiality requirement.

278 (d) Nothing in this section shall compel any health care facility or agency to disclose
279 information in violation of federal law or to waive privileges or protections otherwise provided
280 by state or federal law.